Anderson Diagnostic Services Private Limited
No 26, Rajarathinam Street, Kilpauk, Chennai - 600 010.
Helpline No: 044-4353 9444 / 99620 00048
Email: enquiries@andersondiagnostics.com
www.andersondiagnostics.com

Peripheral Blood Karyotyping

Patient name	: **Mr. Parthiban (UMR2602047090)**	**PIN**	: **AND25150125705**
Gender/ Age	: **Male / 32 Years**	**Sample Number**	: **1525117237**
Specimen	: **Peripheral blood**	**Sample collection date**	: **16-02-2026**
Referring Clinician	: **Dr. Aparna Das**	**Sample receipt date**	: **17-02-2026**
Hospital/Clinic	: **A4 Fertility Centre - Tambaram**	**Report Date**	: **06-03-2026**

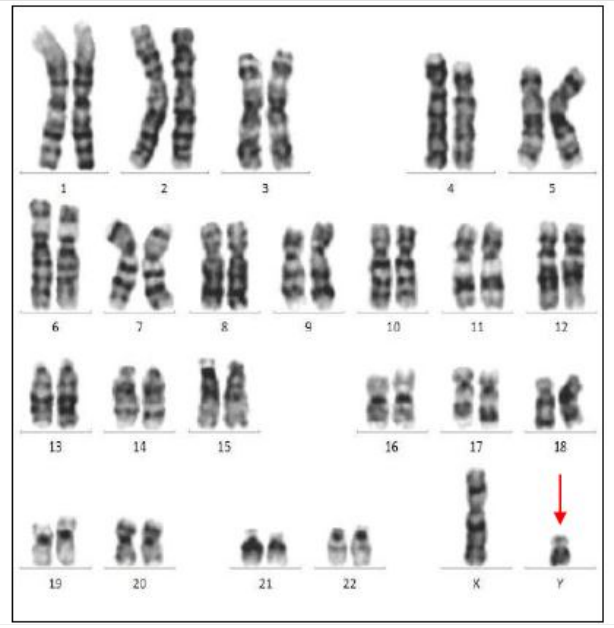
MC-6558

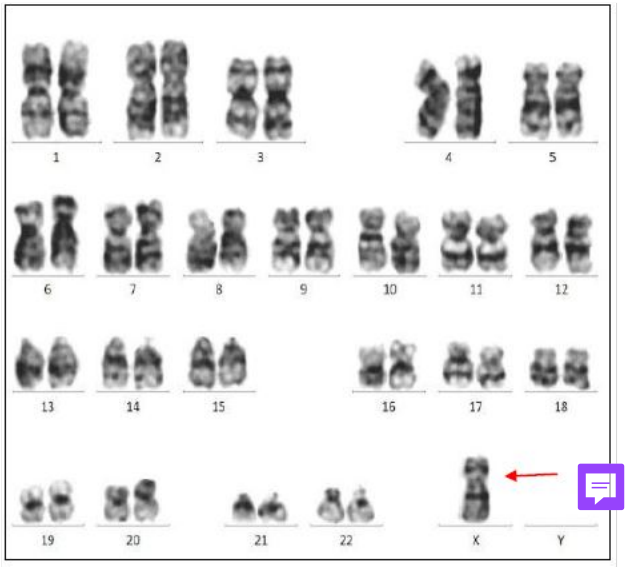
Test Indication

To rule out gross chromosomal abnormality

Result

International System for Human Cytogenomic Nomenclature (ISCN 2024): mos 46,X,del(Y)(q12)[43]/45,X[7]





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PGT-A, PGT-M & PGT-SR | Clinical Exome Sequencing | Microarray | Male Infertility | Recurrent Pregnancy Loss | Carrier Screening | Amniotic Fluid Testing | POC Analysis | NIPS
Fertility Genetics | Genetic Counseling | Oncogenetics | Neurogenetics | Infectious Genetics | New Born Screening

Metaphase analysed : 50	Autosome : Normal
Estimated band resolution : 475	Sex chromosome : Abnormal

Interpretation

Karyotype analysis showed the presence of two cell lines: 86% cells with a large terminal region of the Y chromosome and 14% of the cells with the absence of the Y chromosome. This indicates a mosaic karyotype.

Comments

Reports of individuals with this mosaic karyotype have indicated that they can present a wide spectrum of sex phenotypes, including complete masculinization, ambiguous genitalia, or Turner syndrome.

Recommendations

Clinical correlation and genetic counselling.

Test Methodology

A 72-hour PHA-M stimulated culture of the received peripheral blood sample was processed, according to a protocol adapted from the AGT Cytogenetics Laboratory Manual, Third Edition. Numerical and structural chromosomal abnormalities were ruled out at a banding resolution suitable for the referral indication, in accordance with current ISCN guidelines.

Limitations

- All genetic disorders cannot be ruled out by conventional karyotyping.
- The accuracy of the test is about 99%.
- G-banded analysis cannot detect small rearrangements and submicroscopic deletions.
- Low-level mosaicism may not be detected.

References

1. The AGT Cytogenetics Laboratory Manual Third Edition (1997) *Editors: Margaret J. Barch, TuridKnutsen, Jack L. Spurbeck.*
2. ISCN- An International System for Human Cytogenetic Nomenclature (2024) *Editors: Ros J. Hastings, Sarah Moore, Nicole Chia.*

This report has been reviewed and approved by:

Dr. R. Deepika., Ph.D.,
Consultant Cytogeneticist
Anderson Diagnostics & Labs

Dr. Teena Koshy., Ph.D.,
Consultant Cytogeneticist
Anderson Diagnostics & Labs

Dr. Suriya kumar G
Director
Anderson Diagnostics & Labs